Centers for Medicare and Medicaid Services + Assistant Secretary for Technology Policy

Request for Information – Health Technology Ecosystem

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Digital HIE Inc. Response to Health Technology Ecosystem RFI from CMS and ASTP

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Preface

ABOUT DIGITAL HIE INC.

Vachi Info Tech Inc. DBA Digital HIE Inc. was founded in 2018. The Digital HIE principals are subject matter experts, each with over two decades of experience in enterprise healthcare data integration and interoperability projects at local, state, and federal levels.

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RFI Response

PATIENT AND CAREGIVER USE CASE

Patient Needs – PC-2

Do you have easy access to your own and all your loved ones' health information in one location (for example, in a single patient portal or another software system)?

This is a "yes" or "no" question. The short answer is no.

The longer answer is that the current ecosystem of stand-alone patient portals offers utility to patients and relatively easy online access to their health records at multiple health care enterprise locations, but not in a single patient portal, app, or other software system. Instead, their records must be retrieved from separate data silos that are disconnected from each other. As a whole this record keeping environment can be an inconvenient and cumbersome time sink for patients and caregivers.

Patient portals allow a health care enterprise to provide the convenience of secure online customer access to curated patient records as held by each health care organization. By itself this is a good thing. But a single health care enterprise patient portal, whether hosting hundreds, thousands, or millions of patient records, cannot afford the extraordinary expense to build and operate sufficient health data interoperability services to fully align internal patient records with all external records about each patient. The phrase "boiling the ocean" comes to mind.

Patient Needs – PC-5

What can CMS and its partners do to encourage patient and caregiver interest in these digital health products?

Rather than "*encourage patient and caregiver interest*" in digital health products, it is better for CMS to ensure and enable the flow of patient data while protecting patient rights. The agency has many opportunities to incentivize adoption or optimization of evidence based clinical data interoperability, and to encourage best practices in clinical data informatics.

For example, there is a well known gap between the limited data set mandated under USCDI and the many additional electronic health information (EHI) data types that are are not specified in the current USCDI data set and yet arise with most patient encounters

(e.g., images, narrative notes, faxed content, etc.). As a serendipitous example of leadership towards improved clinical data interoperability, an RFP was released this month by SAMHSA that will incentivize and demonstrate community adoption of the expanded USCDI+BH data set.[1] This is a small step in the right direction. Please use this approach to encourage the public and private sector to originate and support useful and meaningful expansion of clinical data interoperability.

Data Access and Integration – PC-8

In your experience, what health data is readily available and valuable to patients or their caregivers or both?

USCDI content helps create limited data set portions of patient encounters that are nominally available with FHIR APIs. But data curated in separate portals can become functionally inaccessible from the time consuming burden of logging to multiple portals and manually reconciling clinical encounters from separate health care enterprises.

See also the following responses to this RFI created by Josh Mandel:

- "Mandate API-Accessible, Computable Full EHI Export, Aligning with Industry Specifications" – *https://joshuamandel.com/cms-rfi-collab/#req_api_ehi_export_argonaut*
- "Steward USCDI Development for Pragmatic Interoperability" – *https://joshuamandel.com/cms-rfi-collab/#req_steward_uscdi_development*

Data Access and Integration – PC-10

How is the Trusted Exchange Framework and Common Agreement (TEFCA) currently helping to advance patient access to health information in the real world?

The question as stated is out of sync with the limited impact of TEFCA on individual patient-initiated access to their own records. A concerted effort by CMS to more fully grasp patient empowerment would help.

See also the following responses to this RFI created by Josh Mandel:

- "Empower Individuals with Transparency and Control Over TEFCA Data Sharing" – *https://joshuamandel.com/cms-rfi-collab/#req_tefca_individual_transparency_control*

[1] https://www.chickasaw.com/USCDI-BH-Pilot-Program

- "Establish a TEFCA Patient-Developer Credential for Comprehensive, Direct Data Access" – *https://joshuamandel.com/cms-rfi-collab/#req_tefca_patient_developer_credential*
- "Mandate a Trustworthy and Accountable Architecture for All TEFCA Individual Access Services (IAS)" – *https://joshuamandel.com/cms-rfi-collab/#req_tefca_trustworthy_ias_architecture*

PROVIDER USE CASE

Digital Health Apps – PR-3

How important is it for healthcare delivery and interoperability in urban and rural areas that all data in an EHR system be accessible for exchange, regardless of storage format (for example, scanned documents, faxed records, lab results, free text notes, structured data fields)?

This question as stated is likely to generate more noise than signal. First, it is critically important for all data in an EHR to be accessible, period. Second, it is critically important for all data about a patient to be in (or accessible from) an EHR, regardless of data format. Simply put, if the EHR is limited to USCDI content, then EHI data gaps will occur. But third, and perhaps more to the point, the negative consequences of EHR documentation burden on professional clinical staff are well known, well studied, and a persistent problem.[2]

See the following response to this RFI created by Josh Mandel:

- "Comprehensive and Performant Data Access" – *https://joshuamandel.com/cms-rfi-collab/#principle_comprehensive_performant_data_access*

PAYER USE CASE

Patient Empowerment – PA-1

What policy or technical limitations do you see in TEFCA? What changes would you suggest to address those limitations? To what degree do you expect these limitations to hinder participation in TEFCA?

See the following responses to this RFI created by Josh Mandel:

[2] Holmgren, et al., "Electronic Health Record Documentation Burden Crowds Out Health Information Exchange Use By Primary Care Physicians." Health Affairs 43:11, November 2024.

- "Patient Primacy and Empowerment" – *https://joshuamandel.com/cms-rfi-collab/#principle_patient_primacy*
- "Open Innovation and Individual Participation" – *https://joshuamandel.com/cms-rfi-collab/#principle_open_innovation_individual_participation*
- "Transparent and Accountable Networks with Federal Oversight" – *https://joshuamandel.com/cms-rfi-collab/#principle_tefca_oversight_accountability*

Nationwide Provider Directory – PA-4

What would be the value to payers of a nationwide provider directory that included FHIR end points and used digital identity credentials?

See the following response to this RFI created by Josh Mandel:

- "Establish Public Foundational Infrastructure for Nationwide Discovery" – *https://joshuamandel.com/cms-rfi-collab/#req_public_discovery_infrastructure*

TECHNOLOGY VENDORS, DATA PROVIDERS, AND NETWORKS USE CASE

Ecosystem – TD-1

What short term (in the next 2 years) and longer-term steps can CMS take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers?

See the footnote in the answer to PC-5 on page 4. CMS can use the same approach to create incentives for pilot implementations that demonstrate incorporation and utilization of EHI data elements not currently in USCDI.

See the following response to this RFI created by Josh Mandel:

- "Foster Competition Through Open and Fair Market Foundations" – *https://joshuamandel.com/cms-rfi-collab/#principle_market_competition_foundations*

Technical Standards and Certification – TD-10

How can CMS better encourage use of open, standards-based, publicly available APIs over proprietary APIs?

The 21st Century Cures Act (2016) envisioned data being accessed, exchanged, and used without special effort over the full life cycle of a patient's interaction with their clinical caregivers. Now would be a good time for CMS to release new incentives that reward demonstrations of actual "without special effort" deployments.

See the following responses to this RFI created by Josh Mandel:

- "Keep Single-Patient API Certification Current with SMART App Launch & Backend Services Specification" – *https://joshuamandel.com/cms-rfi-collab/#req_update_smart_app_launch_cert*
- "Mandate Patient-Initiated Secure Messaging via Standardized APIs" – *https://joshuamandel.com/cms-rfi-collab/#req_patient_app_messaging_argonaut*
- "Mandate Electronic Pathways for Patient Record Amendment Requests" – *https://joshuamandel.com/cms-rfi-collab/#req_patient_amendment_pathways*
- "Ensure Programmatic and Automated Access to Medical Images" – *https://joshuamandel.com/cms-rfi-collab/#req_coordinated_remote_provisioning_access*

Technical Standards and Certification – TD-11

How can CMS better encourage use of open, standards-based, publicly available APIs over proprietary APIs?

See answer to PC-8. The current non-API approach is insufficient. ASTP should mandate alignment with the Argonaut Project EHI Export API IG.[3] CMS should fund production pilots of the IG, then publish an informed update of the IG.

VALUE-BASED CARE USE CASE

Digital Health Adoption – VB-3

What are essential health IT capabilities for value-based care arrangements?

See the following responses to this RFI created by Josh Mandel:

- "Keep Bulk Data API Certification Current with FHIR Bulk Data Specifications" – *https://joshuamandel.com/cms-rfi-collab/#req_update_bulk_data_cert*
- "Ensure Foundational Design and Performance for Bulk Data API" – *https://joshuamandel.com/cms-rfi-collab/#req_ensure_bulk_api_performance_parity*
- "Mandate CDS Hooks for Seamless Clinical Decision Support Integration" – *https://joshuamandel.com/cms-rfi-collab/#req_mandate_cds_hooks*

[3] https://github.com/argonautproject/ehi-api/

- "Mandate FHIR Subscriptions for Event-Driven Workflows" – *https://joshuamandel.com/cms-rfi-collab/#req_mandate_fhir_subscriptions*

Appendix

ACRONYM GLOSSARY

AI – Artificial Intelligence. *See Machine Learning (ML).*

API – Application Programming Interface.

ASTP – Assistant Secretary for Technology Policy. *See ONC.*

BH – Behavioral Health.

CDS – Clinical Decision Support.

CEHRT – Certified EHR Technology. *See EHR.*

CMS – Centers for Medicare and Medicaid Services.

DBA – doing business as.

EHI – Electronic Health Information, all patient care data, not just USCDI. *See USCDI.*

EHR – Electronic Health Records. *See CEHRT.*

FHIR – Fast Healthcare Interoperability Resources, published by HL7. *See HL7.*

HIE – Health Information Exchange.

HL7 – Health Level 7, an ISO. *See ISO.*

IAS – Individual Access Services. *See PC-10.*

IG – Implementation Guide. *See TD-11.*

ISO – International Standards Organization. *See HL7.*

ML – Machine Learning. *See Artificial Intelligence (AI).*

ONC – Original name of HHS division now known as ASTP. *See ASTP.*

PA – Payers, *a use case.*

PC – Patients and Caregivers, *a use case.*

PR – Providers, *a use case.*

QHIN – Qualified Health Information Network. *See TEFCA.*

RFI – Request for Information.

RFP – Request for Proposal.

SMART – Substitutable Medical Applications and Reusable Technology. *See TD-10.*

TD – Technology Vendors, Data Providers, and Networks, *a use case.*

TEFCA – Trusted Exchange Framework and Common Agreement. *See QHIN.*

USCDI – United States Core Data for Interoperability. *See EHI.*

VB – Value-Based Care, *a use case.*